

Thyroidectomy

You may have surgery to remove part or all of the thyroid gland. This is called a thyroidectomy. It will depend on the size of the cancer.

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What is a thyroidectomy?

Surgery is usually the main treatment for [papillary and follicular thyroid cancer](#). During surgery, part or all of the [thyroid gland](#) is removed. This is called a thyroidectomy.

What are the different types of thyroidectomy?

Your doctors will look at the results of your [scans and biopsies](#) to advise you on which operation is best for you. This might mean having either:

- all the thyroid removed – called a total thyroidectomy
- half of the thyroid removed – called a lobectomy or hemithyroidectomy.

If you were diagnosed with [thyroid cancer](#) during surgery for other thyroid problems, you may need further treatment.

Some thyroid doctors think that some very small, low risk thyroid cancers could be monitored with tests rather than removed with surgery straight away. You and your doctor can talk about the possible risks and advantages of this approach. Then you can decide if this is right for you.

Total thyroidectomy

This is when the surgeon removes the whole [thyroid](#). If it is not possible to remove the whole thyroid the surgeon will remove most of it. This is called a near-total thyroidectomy.

If the cancer has started to spread outside the thyroid, the surgeon may also need to remove some of the tissue around it. Your doctor will discuss this with you before the operation.

Lobectomy (hemithyroidectomy)

You might have a lobectomy if:

- the cancer is small and has a low risk of coming back
- repeated tests on the thyroid cells do not clearly show if the cells are cancerous, but your doctor suspects cancer.

The surgeon removes the lobe of the [thyroid](#) containing cancer or where cancer is suspected. The isthmus, which is the tissue between the left and right lobe, may also be removed.

If cancer has not already been confirmed this operation can diagnose it. Doctors sometimes call this a diagnostic lobectomy. It also tells the doctor if you need more surgery to remove all the thyroid.

Surgery to remove the lymph nodes

The most common place for thyroid cancer to spread is to the [lymph nodes](#) around and below or in front of the thyroid. Removing lymph nodes from the neck is called a neck dissection. If you have a small thyroid cancer you might not need to have any lymph nodes removed.

A neck dissection is done to:

- remove suspicious or cancerous lymph nodes
- reduce the risk of the cancer coming back – when the cancer has a high risk of the spreading to the lymph nodes.

Your surgeon may remove the lymph nodes from:

- the side of your neck
- the front of your neck (central neck).
- the side and the front of your neck.

Removing the lymph nodes also gives your doctor information to help them plan further treatment. Your surgeon or nurse will talk with you about the benefits and disadvantages of lymph node surgery before your operation.

Side effects of a thyroidectomy

Thyroid hormones

After a total thyroidectomy you will need to take [thyroid hormone replacement therapy](#) to replace the hormones your thyroid used to make.

After a lobectomy you might not need to take thyroid hormones. The remaining thyroid may produce enough hormones.

Neck stiffness

Your neck may feel stiff and uncomfortable after surgery. This usually gets better after a few weeks. But it may continue for longer if you had surgery to remove some lymph nodes.

If you had a neck dissection you will usually be given exercises to help strengthen your neck and shoulder muscles. You usually do these for up to 3 months after surgery.

You can take painkillers 30 minutes before you start the exercises to make sure you are comfortable and do not have any pain. Your doctor may refer you to a physiotherapist if you have problems with neck stiffness.

We have more information about [managing neck and shoulder problems after treatment](#).

Scarring

After your operation, you will have a scar on the front of your neck. The scar is usually in a natural skin fold. It is usually about 4 to 6cms.

If you have white skin the scar will be red to begin with. If your skin is brown or black the scar will usually be darker than the surrounding skin.

The colour of the scar fades as it heals so that eventually the scar should be much less noticeable.

If you have more surgery to remove lymph nodes you will still have 1 scar, but it will extend up the neck towards your ear.

Your nurse can show you how to gently massage your wound. This can help to help flatten the tissue. They might suggest creams or oils that might also help. We have more information on [managing scars](#).

Hoarse voice

The [thyroid gland](#) is close to the nerves that control your vocal cords. Sometimes, these nerves can be bruised or damaged during surgery. This can make your voice sound hoarse and weak. Your doctor may check your vocal cords before and after your surgery.

A hoarse, weak voice is usually temporary, but may be permanent in a very small number of people. You may be referred to a speech and language therapist for specialist advice.

Change in calcium levels

If your parathyroid glands are damaged, the level of calcium in your blood may become low. This is called hypoparathyroidism. It can cause:

- tingling in your hands or feet, or around your mouth
- unusual muscle movements, such as jerking, twitching, spasms or muscle cramps.

Your doctor or nurse will check the calcium level in your blood after your operation. If it is low, they will give you calcium either as a tablet or through a drip in your arm. They check your levels every day until they improve.

You might need to take calcium tablets for a short time when you go home. After this your GP or hospital doctor will check your calcium levels regularly.

Lymphoedema

When lymph nodes are removed as part of your thyroid operation there is a risk that fluid might build up in the tissues of the neck. This is called [lymphoedema](#).

It is common to get swelling (oedema) near the surgery scar in the first few days after the operation. This can take several weeks to go away completely. This type of swelling is not the same as lymphoedema.

Lymphoedema usually happens a few months or years later. Taking care of the skin in the area can help reduce the risk of lymphoedema.

If you are worried about any swelling, talk to your doctor or specialist nurse straight away.

Getting support

Macmillan is here to support you. If you would like to talk, you can:

- Call the Macmillan Support Line for free on [0808 808 00 00](tel:08088080000).
- [Chat to our cancer information specialists online](#).
- Visit our [Online Community thyroid cancer forum](#) to talk to people who have been affected by thyroid cancer, share your experience, and ask an expert your questions.

Thyroid cancer risk groups

Doctors look at the risk of thyroid cancer coming back (recurrence) during or after treatment ends. This is called dynamic risk stratification (DRS). It includes how the cancer responds to treatment, including [radioactive iodine](#).

Doctors might also use a form of DRS after surgery when they know more about the cancer.

Assessing the risk of the cancer coming back can help you and your doctor to make decisions about:

- how much treatment you need
- if you need to take [thyroid hormones](#).

Doctors decide on the risk group based on:

- the results of a neck [ultrasound scan](#) or radioactive iodine scan

- [blood test for thyroglobulin](#) 6 to 12 months after radioiodine treatment.

They group risk from 1 to 4. Group 1 is the lowest risk with group 4 having a higher risk of the cancer coming back.

Some thyroid cancers that are high risk can become low risk if they respond well to treatment with radioactive iodine.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our thyroid cancer information. If you would like more information about the sources we use, please contact us at

informationproductionteam@macmillan.org.uk

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help